Headache at the Back of the Head

A simple guide to possible causes, safe self-care, and when to seek medical help

Important: This guide is for general information, not a diagnosis. The cause depends on your symptoms, medical history, and examination.

What does it mean?

Pain at the back of the head (the occipital area) can feel like pressure, aching, sharp pain, or pain that spreads from the neck. It may be mild and temporary, but sometimes it can signal a serious problem.

Common possible causes

- **Neck muscle tension or posture:** Long hours looking down at a phone/computer, awkward sleeping position, stress, or tight neck/shoulder muscles.
- **Tension-type headache:** Often feels like pressure or tightness and may involve the neck.
- **Cervicogenic headache:** Pain referred from joints or structures in the neck; neck movement may worsen it.
- **Occipital neuralgia:** Irritation of nerves near the base of the skull; can cause brief shooting, electric, or burning pain and scalp tenderness.
- **Other causes:** Migraine, illness, dehydration, or medication overuse can sometimes cause pain in this area. High blood pressure usually does not cause symptoms unless it is extremely high or there is a complication.

What you can try for mild pain

- Rest in a quiet place and drink water; eat regularly if you have missed meals.
- Take breaks from screens and avoid keeping your head bent forward for a long time.
- Try a warm pack on tight neck/shoulder muscles for 15–20 minutes. Keep it comfortably warm, not hot.
- Use gentle neck and shoulder movement only if it does not worsen the pain. Avoid forceful neck manipulation or cracking.
- Keep a note of when the headache starts, how long it lasts, severity, triggers, and any other symptoms.
- If considering an over-the-counter pain reliever, follow the label and ask a pharmacist/clinician if you are pregnant, have ulcers, kidney/liver disease, take blood thinners, have allergies, or are unsure what is safe. Do not give aspirin to children or teenagers unless a clinician directs it.

Get emergency medical help now if…

The headache is sudden and extremely severe (a “worst-ever” headache), or it comes with weakness/numbness, trouble speaking, confusion, fainting, seizure, new vision loss, difficulty walking, fever with a stiff neck or rash, or follows a significant head injury. Seek urgent help for a severe headache during pregnancy or soon after delivery, too.

Arrange a medical check soon if…

- This is a new or unusual headache, keeps returning, is getting worse, or does not improve.
- You have persistent neck pain, scalp tenderness, vomiting, or pain triggered by coughing/exertion.
- You are over 50 and have a new headache, or you have cancer, a weakened immune system, or take blood-thinning medicine.

What a clinician may check

A clinician may ask when the pain began, its severity and pattern, medicines you take, and associated symptoms. They may check blood pressure, the nervous system, and neck movement. Scans or blood tests are only needed in some situations based on the history and examination.

Questions to help decide the next step

If you seek care, be ready to say: When did it start? Was it sudden or gradual? Is it one-sided or both sides? Is it sharp, throbbing, or pressure-like? Does neck movement change it? Do you have fever, vomiting, vision changes, weakness, or numbness?

This document cannot replace an in-person medical assessment. If you are worried or symptoms are severe, seek medical care.